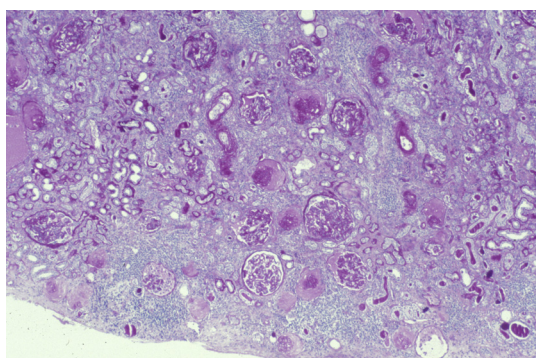


Chronic Allograft Nephropathy

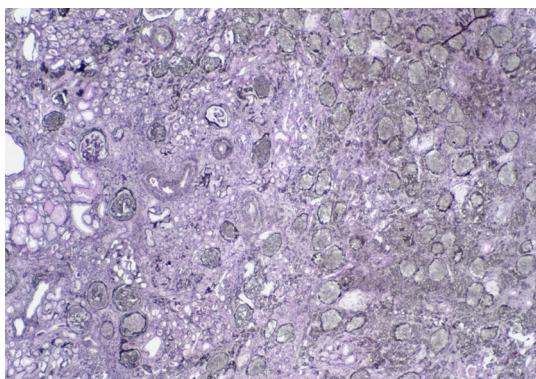
Agnes Fogo, MD

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Fig 1. Chronic allograft nephropathy is a non-specific term, which does not imply pathogenesis but rather, refers to the sum of all fibrotic injuries which affect the graft. There is glomerulosclerosis, interstitial fibrosis, and arterial intimal fibrosis, as illustrated in this transplant nephrectomy (periodic acid Schiff stain, $\times 100$).



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Fig 2. Chronic allograft nephropathy is manifest by widespread global glomerulosclerosis, interstitial fibrosis with proportional tubular atrophy and vascular intimal fibrosis. The lesions are thought to be multifactorial, and include the sum of responses to all injuries directed against the graft, including pre-existing lesions in the donor (Jones' silver stain, $\times 100$).

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Medical Photographer: Brent Weedman.

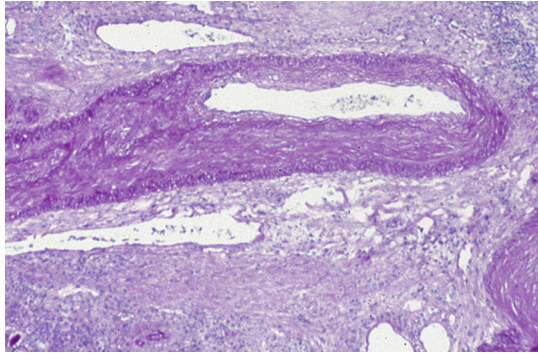
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Am J Kidney Dis. 41(1):E29-E30.

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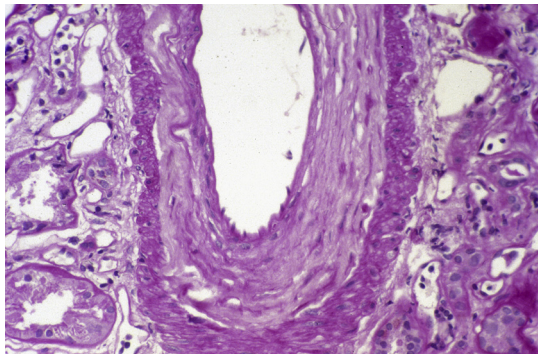
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[http://dx.doi.org/10.1053/S0272-6386\(13\)90051-2](http://dx.doi.org/10.1053/S0272-6386(13)90051-2)



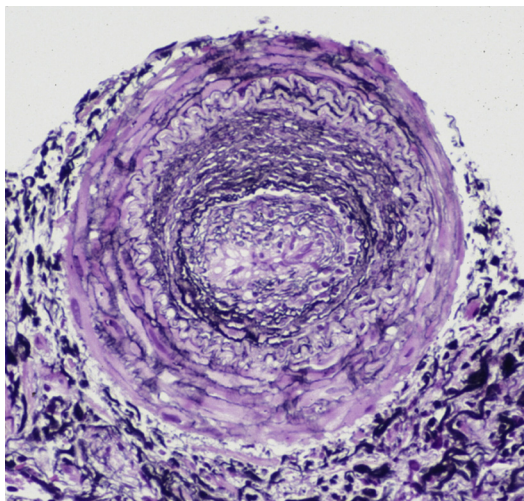
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Fig 3. There is widespread severe arterial intimal fibrosis and medial thickening in this failed renal transplant, characteristic of chronic allograft nephropathy (periodic acid Schiff stain, $\times 200$).



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Fig 4. Chronic allograft nephropathy typically is manifest by intimal fibrosis with concentric re-duplication of the intima, as shown in this large artery (periodic acid Schiff stain, $\times 200$).



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Fig 5. The failed renal allograft often exhibits chronic allograft nephropathy with superimposed acute rejection due to tapering of immunosuppression in anticipation of graft nephrectomy. This is illustrated in this case as severe chronic allograft nephropathy, manifest by concentric reduplication of the intima, with superimposed endothelialitis (Jones' silver stain, $\times 200$).